

MyLoo

for patient advocates and gut-health creators - and what the research says about how to pitch it

"Photo in, AI classifies, clinic PDF out. Privacy-first. Earn from the value you already create."

You replied, thank you. This is the long-form companion to the email. It walks through what the research says about face-on-camera vs faceless content, then breaks down which path fits your creator archetype.

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1. App + business in 60 seconds

MyLoo is a digestion / stool tracker for iOS (Android on hold - Google Health Policy 2026). Photo of the sample, Claude Vision AI classifies on the Bristol Stool Scale 1-7. Privacy-first: photos NOT stored, only structured result lives in timeline. Symptom tags, 30-day PDF for GI appointment.

Why it sells

Word-doc tracking is reality for 80% of IBS patients. MyLoo solves exactly that: at the GI appointment you show a PDF with real data instead of talking retrospectively for 4 weeks.

Free vs Premium

- Free: manual Bristol entry + symptom tags, timeline, manual PDF export.
- Premium (4.99 USD/m or 29.99 USD/y): AI photo classification, email-inbound parsing, extended export.
- Free Lifetime Premium for onboarded affiliates.

2. UGC vs faceless - what the research says

Face-on-camera content (UGC) converts higher than faceless content on average. Faceless still works - just differently.

Two findings from peer-reviewed influencer-marketing research are worth knowing. First, Sokolova and Kefi (2020) showed that parasocial interaction - the sense that a creator is a friend you happen to know through your phone - is a strong driver of purchase intent on Instagram and YouTube. Parasocial trust builds fastest when audiences see a face, hear a voice, and watch a person make choices. Second, Lou and Yuan (2019) found that perceived creator credibility (trustworthiness, expertise, attractiveness) directly increases consumer trust in branded content; creator visibility is one of the strongest credibility signals. Industry data from the Influencer Marketing Hub annual benchmarks consistently puts face-on-camera Reels and TikToks at higher engagement than pure screen-recordings, though the gap varies a lot by niche.

The caveat

Faceless has real strengths. When the demonstration is self-explanatory - a tool doing the thing, a satisfying B-roll, a screen-recording that compresses a 20-minute task into 30 seconds - the product replaces the personality. Faceless also scales: lower production cost per post, no on-camera nerves, easier to outsource. The cost is parasocial trust, which means lower conversion per view but often higher reach.

When each path works

UGC fits when face-on-camera is your credibility asset (you, your craft, your daily reality), when trust matters more than reach, and when you'd naturally say "I personally use this". Faceless fits when the demonstration carries itself (product on screen does the work), when you want to scale output, when on-camera isn't an option, or when you post under a brand persona rather than a personal one.

Sources

- Sokolova, K., & Kefi, H. (2020). Instagram and YouTube bloggers promote it, why should I buy? How credibility and parasocial interaction influence purchase intentions. *Journal of Retailing and Consumer Services*, 53, 101742.
- Lou, C., & Yuan, S. (2019). Influencer marketing: How message value and credibility affect consumer trust of branded content on social media. *Journal of Interactive Advertising*, 19(1), 58-73.
- Influencer Marketing Hub. (2024). *The State of Influencer Marketing Benchmark Report 2024*.
- HypeAuditor. (2024). *State of Influencer Marketing Report*.

3. Creator types - which path fits you

For MyLoo specifically, lived-experience creators convert dramatically higher than aesthetic accounts - health audiences see through performance.

Creator type	Primary path	Best hook angles	Realistic conversion
IBS / IBD day-in-the-life	UGC strong	Word-doc moment; "how I prep for my GI appointment"; 30-day PDF reveal; "finally a tool I trust"	3-5%, very high trust + share rate
Low-FODMAP / food-intolerance	UGC + Faceless mix	Food-symptom correlation timeline; "what I ate vs how I felt"; trigger-discovery story	2-4% in elimination-diet niches
Patient advocate / IBS-IBD educator	UGC	Educational explainer with personal data; "why the Bristol scale matters"; "what to bring to your GI"	2-4%, very high authority
Healthcare professional in personal capacity	UGC (with disclosure)	"What I wish my patients tracked"; clinical-grade tool walkthrough; "the PDF I want at appointments"	2-4% but respect professional regulations
Anonymous gut-health TikTok	Faceless	Silent Bristol-scale education; symptom-tracking voiceless tutorial; privacy-first photo handling demo	1-2% per view, high posting cadence
Post-surgery / chronic-condition	UGC primary	Recovery timeline; "what helped me feel heard at appointments"; structured tracking value	2-3% in chronic-condition communities

4. UGC strategy (you on camera)

Who this fits

You with the condition. IBS patient sharing day-in-the-life. Post-surgery person tracking recovery. Patient advocate with credibility your audience trusts. Health-condition content sees through performance - your lived experience is the entire credibility model.

Hook examples that work

- "This replaced my Word doc" - talking head with real 4-week Word doc, then MyLoo timeline.
- "How I prep for my GI appointment now" - calm 60s, ending with PDF export.
- "Year one of IBS - what I wish I'd tracked from day one" - retrospective.
- "My GI doc actually engaged with the data this time" - post-appointment debrief.
- "AI classified what I couldn't bring myself to look at" - privacy angle.
- "What changed in 30 days of tracking" - pattern-discovery.
- "Why the Bristol scale matters and why nobody told you" - educational.
- "My low-FODMAP elimination week - what the timeline showed" - food correlation.

Format templates

- Talking head 60s.
- Day-in-the-life 90s.
- Educational vlog 75s.
- Emotional payoff reel: post-appointment debrief.
- Retrospective: "year one with IBS".
- Comparison: Word doc vs MyLoo.

Do / Don't

- Do: only post if you actually have the condition or work in the space.
- Do: name your condition or context specifically.
- Do: discuss privacy honestly.
- Don't: give medical advice.
- Don't: medicalise the marketing.
- Don't: oversell the AI.

5. Faceless strategy (screen-rec, voiceover, B-roll)

Who this fits

Anonymous patient educator. Healthcare professional in personal capacity. Voiceover-only chronic-condition account. Many gut-health TikTok creators are intentionally anonymous because privacy is part of their value.

Hook examples that work

- Silent Bristol-scale infographic carousel.
- 30s screen-rec of AI classification with voiceover.
- "Privacy-first photo handling - what 'not stored' actually means" - explainer carousel.
- "What to bring to your first GI appointment" - tutorial carousel.
- "5 apps that actually help with IBS" - listicle, MyLoo at #1-2.
- Symptom-tracking time-lapse - anonymous hand.
- "The Bristol scale - three things most patients don't know" - silent reel.
- Comparison: Word-doc-vs-MyLoo split-screen.

Format templates

- Educational carousel: 6 slides, calm clinical aesthetic.
- Screen-rec demo 20-30s.
- Voiceover narrative 45s.
- Listicle.
- Tutorial: step-by-step setup.
- Silent comparison.

Do / Don't

- Do: clinical-calm tone - MyLoo's audience-fit is patients who hate wellness aesthetics.
- Do: caption every step.
- Do: cite credible sources (NHS, Mayo Clinic, Rome IV) when teaching.
- Do: link in caption + bio.
- Don't: bright wellness-aesthetic colours.
- Don't: pretend to be a patient if you're not.

6. Audience fit

Works well for

- IBS / IBD day-in-the-life creators.
- Low-FODMAP food channels.
- Patient-advocate IBS/IBD pages.
- Post-surgery / chronic-condition journals.
- Anonymous gut-health TikTok (privacy as brand).
- Healthcare professionals in personal capacity.

Works poorly for

- Wellness-influencer aesthetic.
- Diagnostic / 'AI tells you what's wrong' angle.
- General lifestyle accounts without real gut-health connection.

7. Posting cadence + best times

Gut-health: Sunday evening + Wednesday evening. IBS day-in-the-life: Tue-Thu morning. Patient-advocate: late evening 9-11pm. 1 reel per week. Stories with link after GI appointments drive conversion.

8. Conversion math (why this pays)

50% rev share, 24 months. Premium 4.99 USD/m or 29.99 USD/y. IBS day-in-the-life micro (40k followers): 20,000 views x 2.5% CTR = 500 clicks, 40% install = 200 installs, 6% Premium = 12 subs, ~180 USD per reel. Patient-advocate mid-tier (100k): 50,000 views x 2% CTR = 1,000 clicks, 35% install = 350 installs, 5% Premium = 17 subs, ~255 USD. Anonymous gut-health macro (300k TT): 150,000 views x 1% CTR = 1,500 clicks, 30% install = 450 installs, 4% Premium = 18 subs, ~270 USD. Chronic-condition users keep subs longer than average, so 24-month attribution pays out fully more often.

9. Tracking summary (full walkthrough in the Extended Brief)

Your link [myloo.org/i/\[yourhandle\]](https://myloo.org/i/[yourhandle]) works on every platform. Android uses Google install-referrer (deterministic). iOS warm uses Universal Link (deterministic). iOS cold (~99% of creator traffic) uses our DIY signed-token clipboard deferred-deeplink, live-verified, no Branch.io. RevenueCat fires on Premium purchase and credits 50% of net revenue to your handle for 24 months from each user's first paid month. 30-day refund holdback before payout becomes claimable. Full walkthrough is in the Extended Brief PDF.

10. Onboarding (5 minutes)

- Click the personal link from Mail 2 (valid 7 days).
- Land on [myloo.org/affiliate/\[your-token\]](https://myloo.org/affiliate/[your-token]).
- Fill in: handle, Wise payout details, country (for tax docs), billing address.
- Get your personal tracking link by mail + dashboard unlock.
- Free Lifetime Premium activation. No posting obligation, no expiry.

11. Dashboard (what you see)

- Live at getklar.org/dashboard (magic-link login).
- Per day / week / month: clicks, installs split (Android / iOS warm / iOS cold), active Premium subs in your bucket.
- Claimable EUR + paid EUR + next payout date.
- Refund-holdback pipeline + per-app earnings funnel.

12. Brand assets (download)

Logo, app screenshots, Bristol-scale educational slides (medically reviewed), clinical-calm visual identity and ready-made captions: getklar.org/i/myloo/brand-assets.

13. FAQ

I'm a healthcare professional, can I post about MyLoo?

Yes, in personal capacity with disclosure. Frame as 'a tool patients can use', not 'a tool that diagnoses'. Follow your local regulatory rules.

What about Android?

Built and tested but paused at Google's Health Policy step (corporate-account + D-U-N-S required). Your link works for Android - waitlist page. Attribution applies retroactively when Android relaunches.

Can I show real photos?

No. Privacy-first means photos are never stored - that's the trust unlock. Use structured-result view (timeline, Bristol, tags).

Disclosure?

Yes, rev-share = paid recommendation. Plus: healthcare professionals follow local conduct rules.

Flat fee?

Mid-tier \$150-500, macros \$500+. Rev share stays on top.

What if I lose niche credibility?

Don't post if MyLoo doesn't fit your lived experience or professional context. We'd rather miss the post than damage credibility.

14. One last thing

Honest read: MyLoo fits creators with real gut-health experience or professional context. If you don't have IBS, don't work in healthcare, and your audience isn't in a health niche, this is not a fit and we'd rather you not post. Patient audiences see through performance. Reply to Mail 2 if you want to talk - yes, no, or 'maybe, let's see'.

On posts with affiliate link: #ad / #sponsored / Paid Partnership (FTC, EU disclosure law).